

14 DAY WAIT – URGENT REFERRAL**ROYAL STOKE UNIVERSITY HOSPITAL****BRAIN AND CNS**

General Enquiries: Tel 01782671117

Clinical Enquiries: Tel 01782 675272

ALL REFERRALS TO BE SENT VIA RAS (REFERRAL ASSESSMENT SERVICE)**YOU ARE NO LONGER REQUIRED TO EMAIL/FAX UBRN**

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July 2017

PATIENT DETAILS		GP DETAILS	
Name:	Given Name Surname	Name:	Registered GP Title Registered GP Forenames Registered GP Surname
Address:	Home Full Address (single line)	Address:	Organisation Name, Organisation Full Address (single line)
NHS Number:	NHS Number	Phone No:	Organisation Telephone Number
Hospital Number:	Hospital Number	Name of referrer:	
Date of Birth:	Date of Birth	Decision to refer date:	Short date letter merged
Sex:	☐ Male ☐ Female	Interpreter required:	☐ Yes ☐ No
Sign Language required:	☐ Yes ☐ No	Language:	Main Language
Contact No (next 48 hrs):	Home: Patient Home Telephone	Work: Patient Work Telephone	Mobile: Patient Mobile Telephone

BEFORE REFERRAL

The initial Brain pathway is **urgent direct access to CT** (not MRI) which is bookable through UHNM ICE/Radiology referrals- please give as much clinical information as possible to ensure that this is triaged appropriately. Referral is appropriate for a patient whose symptoms meet the criteria outlined below on this form. **This form is not a CT request form.**

On receipt of the GP request for a CT scan for Suspected Brain and CNS Cancer the imaging department will initiate a CT appointment and once performed the report will be written on the same day and faxed back to the referring GP.

The target for this whole process is 14 days.

If the CT scan report indicates the need for an "Urgent Cancer Referral" the GP should then complete and send this Brain 14 day wait urgent referral form along with a copy of the CT report to the Cancer Referral Bureau, who will ensure the patient is seen within 2 weeks.

REASON FOR REFERRAL (please indicate all that apply)**CNS symptoms (for suspected brain tumour)**

- | | | | |
|----------------------------------|--------------------------|-----------------------------------|--------------------------|
| Progressive neurological deficit | ☐ | New onset seizures | ☐ |
| Headaches | ☐ | Mental changes | ☐ |
| Cranial nerve palsy | ☐ | Unilateral sensorineural deafness | ☐ |

Headache

- | | |
|--|--------------------------|
| New unexplained headache in someone who does not normally have headaches | ☐ |
| Headache of an altered nature/qualitatively different headache in someone who has a history of headaches | ☐ |

Recent onset of the above with the following features suggestive of raised intracranial pressure

- | | | | |
|--|--------------------------|--|--------------------------|
| Vomiting | ☐ | Drowsiness | ☐ |
| Posture-related headache | ☐ | Blackouts | ☐ |
| Papilloedema | ☐ | Headache with pulse synchronous tinnitus | ☐ |
| Qualitatively different, unexplained headache, becoming progressively severe and of up to 2 weeks duration | ☐ | | |

Rapid progression of

- | | | | |
|-------------------------------------|--------------------------|---|--------------------------|
| Subacute focal neurological deficit | ☐ | Unexplained cognitive impairment | ☐ |
| Unexplained behavioural disturbance | ☐ | Personality change confirmed by a witness | ☐ |

Previous Investigations

- | | | |
|--|--------------------------|---|
| CT/MRI of Brain suggestive of malignancy | ☐ | Date and place of scan (please attach report) |
|--|--------------------------|---|

☐ I confirm that I have discussed the possibility that the diagnosis may be cancer and they may wish to take a family member or significant other to the initial appointment.

☐ I confirm that the patient is available to attend outpatients within the next 14 days

If patient is not available for the next 2 weeks, and aware of nature of referral, consider deferring the referral and seeing the patient again to reassess symptoms

Relevant Test Results (where available)			
Full Blood Count (FBC)	Single Code Entry: FBC - full blood count	Liver Function Test	Single Code Entry: LFT - Liver function test
U&E	Single Code Entry: Urea and electrolytes level	eGFR	Single Code Entry: GFR (glomerular filtration rate) calculated by abbreviated Modification of Diet in Renal Disease Study Group calculation...
Smoking Status	SEE BELOW	Alcohol Status	SEE BELOW

Smoking Status

Smoking

Alcohol Consumption

Alcohol Consumption

Allergies

Allergies

Medication:

Medication

Relevant Medical History

Problems